

KARAMOJA PEDIATRIC ORTHOPEDIC SURGICAL CAMP REPORT

*Specialized Pediatric Orthopedic Camp Outreach in Napak District,
Karamoja Region*

Date of Camp:	Monday 20th July – Friday 25th July, 2026
Host Location:	St. Kizito Hospital Matany, Napak District, Karamoja Region, Uganda
Visiting Surgical Team:	Mbarara Regional Referral Hospital (MRRH) Orthopedic Department
Partner Entities:	Amigos Internationales Bethany Kids Doctors on Mission

EXECUTIVE SUMMARY & MISSION OVERVIEW

Between Monday 20th and Friday 25th July 2026, a specialized orthopedics team from Mbarara Regional Referral Hospital (MRRH) partnered with St. Kizito Hospital Matany in Napak District, Karamoja region, to conduct an intensive 5-day pediatric orthopedic camp. Sponsored by Amigos Internationales and executed in collaboration with Bethany Kids and Doctors on Mission, the outreach successfully attended to 27 orthopedic patients and performed 26 major surgical and non-surgical orthopedic procedures. The camp achieved a 100% surgical success rate with zero intraoperative or post-operative mortalities, resolving life-altering deformities, chronic bone infections, and severe trauma in one of Uganda's most medically underserved sub-regions.

1. Background

The Karamoja sub-region in North-Eastern Uganda faces profound structural challenges in healthcare access, particularly regarding specialized surgical care. Physical trauma, neglected congenital deformities, post-infectious skeletal malformations, and chronic osteomyelitis are widely prevalent among pediatric and adolescent populations. Due to geographic isolation, socio-economic deprivation, and a complete absence of resident pediatric orthopedic surgeons in Napak District, vulnerable children often suffer from severe, progressive disability that hinders mobility, education, and social integration.

To bridge this critical service gap, a collaborative clinical outreach was conducted. The specialized orthopedic surgical team traveled from Mbarara Regional Referral Hospital (MRRH) to St. Kizito Hospital Matany. This mission was made possible through the generous financial sponsorship of Amigos Internationales, alongside essential technical, logistical, and manpower support from Bethany Kids and Doctors on Mission.

1.1 Key Stakeholder Roles

Mbarara Regional Referral Hospital (MRRH)	Provided the primary surgical team, including an orthopedic surgeon, resident surgeons, doctors, anesthesiologists, orthopedic officers and theater nurses; led clinical care and surgical execution.
St. Kizito Hospital Matany (Napak)	Served as the host medical facility, providing operating theater infrastructure, inpatient ward accommodation, local patient mobilization, C-arm fluoroscopy access, radiology services and long-term ward nursing care.
Amigos Internationales	Served as the primary financial sponsor, funding logistical transport, medical/surgical consumables.
Bethany Kids & Doctors on Mission	Acted as key collaborating partners, offering specialized pediatric orthopedic expertise, clinical co-sponsorship, and perioperative clinical staff.

2. Key Objectives Achieved

The surgical camp was guided by clear clinical, operational, and humanitarian goals. Through synchronized effort, the team fully met or exceeded the set goals:

- **Provide High-Quality Orthopedic Interventions:** Completed 26 clinical procedures (20 complex theater surgeries and 6 major non-operative/POP procedures) over 5 operational days.
- **Address Neglected Pediatric Musculoskeletal Deformities:** Successfully corrected severe angular lower limb deformities (Genu Valgum via open wedge osteotomy with locking plates/cross pinning) and complex foot deformities (Resistant Clubfoot via triple arthrodesis and serial casting).
- **Execute Complex Joint Reconstructions & Dislocation Reductions:** Performed open reduction and internal fixation (ORIF) for neglected bilateral congenital hip dislocations in toddlers, as well as fluoroscopy-guided reduction for chronic shoulder and hip dislocations.
- **Treat Advanced Bone Infections:** Executed thorough sequestrectomies and surgical debridements for chronic osteomyelitis (tibia and humerus) to prevent limb loss and systemic sepsis.
- **Provide Urgent & Complex Trauma Management:** Delivered definitive surgical fixation (ORIF with DCP plates, cannulated lag screws, and external fixators) for severe femoral, tibial, and hip fractures in adolescents and adults.
- **Knowledge Transfer & Capacity Building:** Trained local Matany Hospital clinical staff of doctors and nurses in specialized post-operative care, spica casting and traction setup

3. Operational Statistics

A total of 27 unique patients received care during the camp, generating 26 distinct procedural encounters. The demographic and diagnostic profile reflects a high burden of neglected congenital conditions, infection, and severe trauma.

26	27	1	100%	0
Total Procedures Completed	Unique Patients Treated	Referred for specialized care	Surgical Success Rate	Mortality / Major Complications

3.1 Diagnostic Category Breakdown

Diagnostic Category	Patient Count	Percentage of Total Burden
Angular deformities (Genu Valgum)	3 patients	11%
Fractures (Traumatic & Pathologic)	8 patients	30%
Hip dislocations with AVN(Traumatic & Pathe’s disease)	4 patients	15%
Chronic Bone Infections (Osteomyelitis / Sequestrectomy)	2 patients	7%
Congenital Neglected hip Joint/femur Pathology	2 patients	7%
Foot Deformities (Clubfoot)	2 patients	7%
Ankle deformities	1 patient	4%
Elbow & Shoulder deformities	2 patients	7%
Spinal kyphosis	1 patient	4%
Knee osteo-arthritis	1 patient	4%
Cerebral palsy	1 patient	4%

3.2 Daily Surgical & Clinical Volume

Date	Number of patients seen	Surgeries & Procedures Performed	Daily Total procedure Cases
Monday 20th July	12 patients	4 Major Operative Cases (Genu Valgum, Femur ORIF, Sequestrectomy, ExFix)	4 Cases
Tuesday 21st July	2 patients	4 Operative Cases (Ankle Fusion, Revascularization, Girdlestone) + 3 POP Room Cases	7 Cases
Wednesday 22nd July	5 patients	4 Major Operative Cases (Girdlestone Excision, Triple Arthrodesis, Sequestrectomy, Femur ORIF)	4 Cases
Thursday 23rd July	7 patients	4 Major Operative Cases (Theatre Dressing, Genu Valgum ORIF, Bilateral CDH ORIF, Femur OREF)	4 Cases
Friday 24th July	1 patient	5 Major Operative Cases (Genu Valgum, Shoulder Dislocation, 2 NOF/Intertroch ORIFs, Dressing) + 3 Post-Op Casts	7 Procedures

4. Clinical Highlights & Advanced Surgical Management

The camp successfully managed highly complex clinical scenarios requiring advanced technique, specialized hardware, and multi-disciplinary perioperative planning:

Neglected Bilateral Congenital Dislocation of the Hip (Lodio Ethan Jasper, 3.75 yrs):

Presented with severe bilateral hip dysplasia and high dislocation. The surgical team executed bilateral open reductions, clearing intra-articular pulvinar tissue, releasing tight capsular structures, and securing stability with K-wires. The patient was stabilized in a bilateral lower limb hip spica cast in 30-40° abduction using a rigid cross-bar, achieving successful concentric reduction.

Bilateral & Unilateral Genu Valgum Corrections (Akol Agnes 16yrs, Apio Zipporah 16yrs, Iriama Ivan 15yrs):

Severe knock-knee deformities severely impaired gait and joint. Medial open wedge osteotomies were performed. In older adolescents, precise anatomical axis realignment was achieved using contoured L-shaped locking plates and compression screws. In younger cases, cross-pinning combined with above-knee P.O.P casting ensured rigid alignment and minimal soft tissue trauma.

Complex Adult & Adolescent Trauma Fixation:

High-energy trauma cases were managed using image guidance. A basal neck of femur (NOF) fracture (Omura Basil, 49yrs) and a right intertrochanteric fracture (Logiel Apanyethil, 45yrs) were successfully stabilized using cannulated lag screws. Additionally, a distal third femoral shaft fracture was stabilized using an Open Reduction and External Fixator (OREF) with Schanz pins.

Chronic Osteomyelitis & Sequestrectomy (Angella Andrew 12.5yrs, Lopuwa Daniel 7yrs):

Long-standing bone infections with necrotic sequestra and sinus tracts were aggressively treated via longitudinal approaches, curettage of infected bone, copious sterile normal saline irrigation, and staged theatre dressings, effectively eradicating infectious foci and paving the way for bone remodeling.

Resistant Clubfoot & Complex Ankle Arthrodesis (Aleper Esther Faith 14yrs, Lokong Israel 12yrs):

Neglected rigid foot deformities were corrected via Triple Arthrodesis (fusing subtalar, calcaneocuboid, and talonavicular joints) and corrective ankle osteotomy with fusion, restoring a plantigrade, pain-free weight-bearing foot.

5. Challenges & Mitigation strategies

Operating in a remote sub-regional environment posed significant logistical and clinical challenges. The joint team successfully implemented real-time solutions to ensure uninterrupted patient care:

- **High Burden of Late-Stage / Neglected Disease:**

Challenge: Patients presented with extremely advanced structural deformities, stiff unreduced dislocations, and extensive bone necrosis due to delayed presentation.

Mitigation: Senior surgical consultants conducted rigorous pre-operative planning, utilizing imaging and tailored surgical approaches to achieve safety and efficacy.

- **Equipment & Implant Size Constraints:**

Challenge: Specialized pediatric hardware (e.g., small-diameter locking plates, cannulated screw sets, and pediatric power tools) were in limited supply.

Mitigation: The team combined MRRH mobile equipment sets with Matany Hospital inventory and deployed versatile fixation methods such as cross-pinning, external fixators, and specialized P.O.P spica molding.

- **Socio-Economic & Language Barriers:**

Challenge: Communication with Karamojong-speaking guardians regarding non-weight-bearing instructions and post-operative casting hygiene was difficult.

Mitigation: Matany Hospital resident nurses and translators were integrated into daily ward rounds to deliver comprehensive caregiver education in local languages.

- **Post-Discharge Transportation & Rehabilitation Infrastructure:**

Challenge: Many families travel vast distances across rugged terrain, making early outpatient follow-up and physiotherapy challenging.

Mitigation: Detailed discharge care plans, custom crutches/walking frames, and structured referral links with local community health officers and Matany outpatient clinic were established.

6. Recommendations

To maximize the long-term impact of future orthopedic outreach camps in Karamoja, the reporting team submits the following actionable recommendations for consideration by supervisors, donors, and host leadership:

1. **Institutionalize Bi-Annual Orthopedic Outreach Camps:** Given the massive unmet demand, establish a regular bi-annual schedule (e.g., July and January) funded through sustained partnership between Amigos Internationales, Bethany Kids, and MRRH.
- **Pre-Camp Community Screening & Triage:** Deploy local Matany outreach teams 3-4 weeks prior to camp dates to pre-screen, X-ray, and schedule complex cases, reducing initial theater delay and optimizing daily output.
- **Procurement of Dedicated Pediatric Hardware Consumables:** Establish a dedicated 'Karamoja Pediatric Implant Kit' containing small-fragment locking plates, pediatric cannulated screws, modular external fixators, and specialized casting materials.
- **Strengthen Local Physiotherapy & Rehabilitation Capacity:** Conduct formal training workshops for Matany physiotherapy staff during the camp to ensure seamless long-term rehabilitation for casted and operated pediatric patients.
- **Tele-Orthopedic Follow-Up Portal:** Create a digital image-sharing protocol (e.g., WhatsApp/EAFYA HMIS) between Matany medical officers and MRRH consultants to review post-op X-rays at 6 and 12 weeks' post-surgery.
- **Human resource:** Train more orthopedic surgeons, officers, nurses through fellowships, training programs, sponsorships to combat staffing gaps and increase patient care and outcomes especially in high output camps

7. Annex: Complete Surgical & Procedure Log (July 20th –25th, 2026)

The following log details all 26 clinical procedures performed during the Karamoja Mini-Orthopedic Camp at St. Kizito Hospital Matany:

Date	Patient Name & ID	Age/Sex	Diagnosis	Operation Done	Post-Op Outcome / Management
Mon 20 Jul	Akol Agnes UG-KEP-418	16yrs / F	Genu valgum Rt leg	Open Wedge Osteotomy with plating	IV ABGs + analgesia, Crutches NWB x 6wks, Dressing, Physio
Mon 20 Jul	Opio Isaac UG-JUA-834	8yrs / M	Rt Femur shaft fracture	ORIF + Plating with DCP	IV ABGs + analgesia, Crutches NWB x 6wks, Dressing, Physio
Mon 20 Jul	Angella Andrew UG-SYL-663	12.5yrs / M	Rt tibia chronic osteomyelitis	Sequestrectomy Rt leg	IV ABGs + analgesia, Alternate wound dressing
Mon 20 Jul	Angella Andrew UG-SYL-663	12.5yrs / M	Mal-united pathological fracture Lt distal femur	Corrective osteotomy Lt distal femur + ExFix	IV ABGs + analgesia, Alternate wound dressing, Physio
Tue 21 Jul	Lokong Israel UG-XKV-928	12yrs / M	Acquired deformity Rt ankle post infection	Corrective osteotomy with ankle fusion	B/K POP cast, IV ABGs + analgesia, Crutches NWB x 6wks
Tue 21 Jul	Lemukol Benjamin UG-BNZ-883	7yrs / M	AVN of Rt femoral head	Revascularization of femoral head	IV ABGs + analgesia, Crutches NWB x 6wks
Tue 21 Jul	Acedu Patrick UG-RLR-577	8yrs / M	Old post traumatic hip dislocation with AVN	Girdle stone	IV ABGs + analgesia, Skin traction
Tue 21 Jul	Moru Blessing UG-ZJN-129	4yrs / F	Rt Proximal humeral shaft fracture	U-Cast application (POP)	Analgesics, Collar & Cuff sling
Tue 21 Jul	Lokwang John UG-ZFN-829	18yrs / M	Healing fracture tibia midshaft	Patellar Tendon Bearing (PTB) cast	Axillary crutches, partial weight

					bearing
Tue 21 Jul	Lalamu Bernice UG-WBN-474	6yrs / F	Bilateral clubfoot (Talipes)	Lt AFO / cast + Rt B/K POP cast	Serial casting then bracing
Wed 22 Jul	Makole Moses UG-XAN-928	12yrs / M	Chronic hip dislocation with AVN	Excision of femoral head (Girdlestone)	Bilateral lower limb cast in abduction, IV ABGs, Physio
Wed 22 Jul	Aleper Esther Faith UG-YRF-519	14yrs / F	Resistant club foot	Triple Arthrodesis with casting	B/K POP cast, IV ABGs, Crutches NWB x 6wks, Dressing
Wed 22 Jul	Lopuwa Daniel UG-ADN-623	7yrs / M	Rt humeral chronic osteomyelitis	Sequestrectomy	IV ABGs + analgesia, Alternate wound dressing
Wed 22 Jul	Loumo James UG-UYB-479	17yrs / M	Lt femoral fracture midshaft	ORIF + DC Plating	IV ABGs + analgesia, Axillary crutches NWB
Thu 23 Jul	Angella Andrew UG-SYL-663	12.5yrs / M	Chronic osteomyelitis, Lt distal femur fracture	Theatre wound dressing	Alternate wound dressing, IV ABGs, Physio, Crutches
Thu 23 Jul	Apio Zipporah UG-BOA-381	16yrs / F	Genu Valgum Rt leg	Corrective Osteotomy	IV ABGs + analgesia, Crutches NWB x 6wks, Physio
Thu 23 Jul	Lodio Ethan Jasper UG-UKD-162	3.75yrs / M	Neglected bilateral congenital hip dislocation	ORIF Bilateral Hip Dislocation + Cast	Bilateral abduction POP cast, IV ABGs, Physio
Thu 23 Jul	Korengenyang Peter UG-GQW-156	30yrs / M	Post traumatic distal 1/3 femoral shaft fracture	OREF with External Fixation	IV ABGs, Pin dressing, Crutches NWB x 8wks, Physio
Fri 24 Jul	Iriama Ivan UG-UKD-162	15yrs / M	Right Genu Valgum	Corrective osteotomy with cross pinning	Above-Knee POP cast, IV ABGs, Crutches NWB
Fri 24 Jul	Nausi Ngorok UG-HHR-145	56yrs / M	Chronic Right shoulder dislocation	Fluoroscopic reduction, ORIF with	Collar & cuff x 3wks, IV ABGs,

				K-wiring	Physio
Fri 24 Jul	Omura Basil UG-YXL-381	49yrs / M	Basal NOF fracture	ORIF with cannulated screws	IV ABGs + analgesia, Crutches NWB x 6wks
Fri 24 Jul	Logiel Apanyethil UG-ULV-444	45yrs / M	Right intertrochanteric femur fracture	ORIF cannulated screws	IV ABGs + analgesia, Crutches NWB, Physio
Fri 24 Jul	Lopua Daniel UG-ADM-623	7yrs / M	Chronic wound Rt humerus osteomyelitis	Theatre wound debridement & dressing	IV ABGs, Alternate dressing, Collar & cuff sling
Fri 24 Jul	Lodio Ethan Jasper UG-UKD-162	3.75yrs / M	s/p ORIF Bilateral hip dislocation	Bilateral lower limb POP spica (abduction)	Ambulating NWB with crutches / care
Fri 24 Jul	Lokong Israel UG-XKV-928	12yrs / M	s/p Corrective osteotomy & ankle fusion	Wound dressing + B/K POP cast	Non weight bearing on axillary crutches
Fri 24 Jul	Aleper Esther Faith UG-YRF-519	14yrs / F	s/p Triple arthrodesis	Wound dressing + B/K POP cast	Non weight bearing - Issued walking frame

Report Compiled & Submitted By:

Dr Owembabazi Shebah

Volunteer

Doctors on mission international